

• URGENT CNS — TUMOR BOARD PRIORITY

Karen Clayborne — Clinical Decision Summary

Stage IV Lung Adenocarcinoma · EGFR Exon 19 del · Active CNS Metastasis · Prepared for Physician/Tumor Board Review

PATIENT

Karen Clayborne

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CURRENT THERAPY

Tagrisso (osimertinib)

Since May 2025



Active posterior fossa metastasis — brainstem compression confirmed

2.4 cm cerebellar vermis lesion with documented progression despite SRS × 2 and ongoing Tagrisso. Progressive ataxia, dysphagia, left-hand weakness. Steroid-dependent. Delay risks irreversible neurological decline.



TUMOR BOARD SNAPSHOT

ACTIVE LESION

2.4 cm

Cerebellar vermis —
brainstem compression

TRAJECTORY

↑ Progressing

1.0 → 1.4 → 1.9 → 2.4
cm

STEROIDS

6 mg/day

Dexamethasone (up
from 4 mg)

PRIOR SRS COURSES

2 courses

Apr 2025 (5-fx) + Feb
2026 — dominant lesion
NOT re-treated

PRIMARY MUTATION

EGFR Exon 19 del

TEMPUS 2024-10-18

CO-MUTATION

**BRCA2
pathogenic**

Variant confirmed

SYSTEMIC THERAPY

Tagrisso

Osimertinib — since May
2025

SPINAL INVOLVEMENT

T3, T9, L3

+ frontal lesion (6 mm,
Nov 2025)

CEREBELLAR VERMIS LESION — SIZE PROGRESSION

Apr 2025	3.1 × 2.4 × 3.0 cm Initial presentation — 4th ventricle effacement · MRI Report ↗	3.1 cm
Apr–May 2025	SRS delivered — 30 Gy in 5 fractions SRS Plan ↗	Tx
Jun 2025	1.0 × 0.6 × 1.0 cm Post-SRS best response · MRI Report ↗	1.0 cm
Aug 2025	1.4 cm enhancing mass Regrowth — enhancing · MRI Report ↗	1.4 cm
Nov 2025	1.9 × 1.8 cm + new L frontal New left frontal lesion (6 mm) · MRI Report ↗	1.9 cm
Feb 2026	SRS at Mays — 2 smaller lesions only · dominant 2.4 cm NOT treated	Tx
Current	2.4 cm — brainstem compression Active brainstem compression — neurological symptoms progressing · Clinical Note ↗	2.4 cm ⚠

CURRENT NEUROLOGICAL SYMPTOMS



Brainstem Compression

2.4 cm vermis mass with documented mass effect; 4th ventricle involvement on prior imaging



Progressive Gait Ataxia

Worsening, contributed to Jan 2026 hospitalization



New Dysphagia

Emerging symptom consistent with posterior fossa compression



Left-Hand Weakness

Progressive; new neurological deficit



Worsening Headaches

Consistent with increased intracranial pressure



Steroid Dependence

Dexamethasone 6 mg/day (escalated from 4 mg) — symptom control only

IMMEDIATE ACTIONS REQUESTED

01

Urgent Clinical Triage

Expedited review — meets all criteria: progression + brainstem compression + objective neurological decline (probability $\geq 95\%$)

Neuro-Oncology Evaluation

Active CNS disease with brainstem involvement

Neurosurgery Review

Mass effect assessment; surgical candidacy for 2.4 cm lesion

Radiation Oncology — Salvage Options

Post-SRS $\times 2$ progression; dominant lesion not yet re-irradiated at full dose

02

Disease-Site Team Assignment

CNS + Thoracic + RadOnc coordination required (probability $\geq 95\%$)

03

Tumor Board — Expedited (≤ 3 days)

Posterior fossa progression + symptoms = high priority slot (probability $\geq 90\%$)

04

Treatment Plan + Orders

CPT / ICD-10 generation following Tumor Board review (probability $\geq 80-85\%$)

FULL CNS DISEASE TIMELINE — SOURCE DOCUMENTS

DATE	EVENT / FINDING	LESION / DETAIL	SOURCE
Feb–Mar 2025	Initial hospital discovery of cerebellar lesion	~ 3.1 cm	Hospital imaging (pending upload)
Apr 2, 2025	MRI Brain — large vermian mass confirmed	3.1 × 2.4 × 3.0 cm 4th ventricle effacement	 Radiology Report
Apr 3, 2025	MRI Abdomen — spinal metastases	T3, T9, L3 lesions; pleural effusion	 Radiology Report
Apr 10, 2025	SRS plan approved	5-fraction plan — 30 Gy	 SRS Plan
Apr 14–25, 2025	SRS Course 1 delivered	30 Gy / 5 fractions	 SRS Settings
Jun 12, 2025	MRI Brain — post-SRS response	1.0 × 0.6 × 1.0 cm ✓ best	 MRI Report
Aug 8, 2025	MRI Brain — regrowth confirmed	1.4 cm enhancing mass	 MRI Report  M&S Report
Nov 12, 2025	MRI Brain — further growth + new lesion	1.9 × 1.8 cm + new L frontal 6 mm	 MRI Report
Jan 22–30, 2026	Inpatient stay — ataxia, elevated BP	University Hospital admission	 Hospital Summary
Jan 27, 2026	Documented rapid progression	1.4 → 1.9 cm in ~60 days	 Advocate Note
Feb 2026	SRS Course 2 — Mays Cancer Center	1.5 cm + 0.44 cm treated; 2.4 cm dominant NOT treated	Radiation Oncology Notes
Feb 18, 2026	CT Chest/Abd/Pelvis	Vertebral lesions; systemic assessment	 CT Report
Mar 2026	Steroid escalation — symptom-driven	Dexamethasone ↑ to 6 mg/day	 Clinical Note
NOW	Active CNS disease — brainstem compression	2.4 cm 	Pending MRI — urgent repeat needed



MOLECULAR PROFILE

PRIMARY DRIVER

EGFR Exon 19 del

TEMPUS DNA — 2024-10-18 ·



Report

CO-MUTATION

BRCA2 pathogenic variant

Potential therapeutic relevance

LIQUID BIOPSY

Guardant360



Panel 2025-04-23

PATHOLOGY

Lung Adenocarcinoma

Stage IV ·



Report 2025-04-23

CURRENT SYSTEMIC RX

Osimertinib (Tagrisso)

Since May 2025 — CNS still progressing

IMAGING INDEX — ALL STUDIES WITH DIRECT LINKS

BRAIN MRI STUDIES

Apr 2, 2025	MRI	Brain — Initial large vermian mass (3.1 cm)	PDF
Jun 12, 2025	MRI	Brain — Post-SRS best response (1.0 cm)	PDF TX Onc
Aug 8, 2025	MRI	Brain W/W-O Contrast — regrowth (1.4 cm)	PDF M&S DICOM
Nov 12, 2025	MRI	Brain W/W-O Contrast — 1.9 cm + new frontal lesion	PDF M&S DICOM
Jan 16, 2026	MRI	Brain W/W-O Contrast — most recent pre-current	M&S PDF ONLI PDF DICOM
Pending	MRI	Brain — Current (2.4 cm lesion) — URGENT repeat needed	<i>Not yet available</i>

SPINE MRI STUDIES

Apr 3, 2025	MRI	Abdomen — T3, T9, L3 lesions; pleural effusion	PDF
Nov 18, 2025	MRI	Cervical Spine W/W-O Contrast	M&S PDF DICOM
Nov 18, 2025	MRI	Thoracic Spine W/W-O Contrast	M&S PDF DICOM
Nov 18, 2025	MRI	Lumbar Spine W/W-O Contrast	M&S PDF DICOM

CT STUDIES

Jul 26, 2024	CT	Chest W-O IV Contrast	PDF DICOM
Feb 18, 2025	CT	Abdomen / Pelvis / Chest with IV Contrast	M&S PDF TX Onc DICOM
Jul 22, 2025	CT	Chest / Abd / Pelvis W CM	PDF
Aug 8, 2025	CT	Head/Brain W/W-O IV Contrast	PDF DICOM
Oct 15, 2025	CT	Abdomen / Pelvis / Chest with IV Contrast	M&S PDF DICOM
Feb 18, 2026	CT	CT Chest/Abd/Pelvis — systemic assessment	PDF

KEY CLINICAL DOCUMENTS

Oct 2024	DNA	TEMPUS DNA Report — EGFR + BRCA2	PDF
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Apr 2025	PATH	Pathology Report — Lung Adenocarcinoma	PDF
Apr 2025	LBx	Guardant360 Liquid Biopsy Panel	PDF
Jan 2026	HOSP	Hospital Summary — Inpatient Stay	PDF
Mar 2026	REQ	Brain MRI Request / Steroid Escalation Note	PDF
Appendix A	VIS	Patient-Prepared Brain Visuals (orientation only)	Visual 1Visual 2

 HOSPITALIZATION HISTORY	
Jun 17, 2025	UT Health ER Suspected radiation necrosis vs. progression
Nov 9, 2025	Northeast Baptist ER New 6 mm left frontal lesion identified
Jan 22–30, 2026	University Hospital — Inpatient Ataxia, elevated BP ·  Summary

 EMERGENCY CONTACTS
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RECORDS LOCATIONS — KEY CONTACTS FOR DICOM RETRIEVAL

FACILITY	PHONE	FAX	NOTES
Baptist M&S Imaging / M&S Imaging	(210) 785-2590 / (210) 297-7005	210-616-7787	Portal: patient.msris.com · PowerShare capable
University Health — University Hospital	(210) 358-4000	210-358-5936	DICOM: 210-358-8532 · MyChart available
Texas Oncology — SA Medical Center	(210) 595-5300	(210) 656-3687	Contacts: Kary, Zenia · DICOM manually released
Northeast Baptist Hospital	(210) 297-2000	210-297-0217	PowerShare to MD Anderson · Imaging: 210-297-2729
Mays Cancer Center — UT Health MD Anderson	(210) 450-1000	210-450-6058	DICOM burned to CD · 24–48 hr turnaround
STRIC — South Texas Radiology	(210) 617-9000	210-616-7787	DICOM CDs ready — 2 CDs (DICOMDIR format)

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